



POLICY

Admission to Discharge Policy Community Mental Health Services

Scope (Staff):	All NMHS Community Mental Health Services (MHS)
Scope (Area):	All NMHS Community MHS

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1. Aim

To effectively identify admission and discharge processes to facilitate ease of access to community mental health services for consumers, families and carers and provide timely and planned discharge to appropriate services as required in accordance with individual needs and identified risk.

2. Background

The ACSQHC [Standards Guide for Community Health Services](#) requires all community health services need to ensure that the systems and processes in place to undertake the following:

- to develop policy, procedures and processes;
- to identify and manage risk;
- to protect the rights of consumers;

- to promote consumer engagement;

This policy has been developed to provide clinicians with clear and consistent direction as to the requirements regarding the admission, discharge of consumers to and from community mental health services. This policy recognises the requirements of the [DoH Triage to Discharge Mental Health Framework for State-wide Standardised Clinical Documentation \(SSCD\)](#), [the National Safety and Quality Health Service Standard 2 \(Partnering with Consumers\)](#), [Standard 5 \(Comprehensive Care\)](#) and [Standard 6 \(Communicating for Safety\)](#), and the [National Standards for Mental Health Services](#) and incorporates essential elements of these documents to informing an overarching policy directing admissions and discharges Community Mental Health Services (MHS.)

To effectively identify and respond to consumers' clinical needs and to appropriately manage care, in particular for those at increased risk of harm to self or others, all clinical services must have defined admission and discharge processes in place to identify consumers at increased risk of harm, take early action to reduce risks for at-risk consumers, and have systems in place to recognise early clinical deterioration (both physical and mental) and to escalate care in response to clinical deterioration in physical and/or mental health status.

3. Risk

Non-compliance with this policy will impact on the following risk categories:

Culture	☒	Organisational Development	☒
Emergency Management	☒	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☒	Research & Innovation	☐
Information Communications Technology	☒	Staff Safety & Wellness	☒
Facility Management	☐	Workforce Planning	☒
Medical Equipment	☐	Other	☐

4. Definitions

Consumer: A Consumer is a person with lived experience of a mental health condition who is accessing or has previously accessed a mental health service.

Collaborative care: A more holistic and personalised approach to each person allows them to be partners in their own care, to the extent they desire. Collaborative care plans should be meaningful to the person using their own words and phrases, empowering consumers to take ownership in their own care.

Diversity: Diversity is inclusive of but not limited to the diversity among people with respect to ethnicity, religion, spirituality, disability, power, status, gender identity or expression, sexual orientation, neurodiversity and socioeconomic status.

Mental State: Mental state comprises mood, behaviour, orientation, judgement, memory, problem solving ability and contact with reality.

Mental Health Deterioration: Mental Deterioration is identified based on clinical observation and interviewing and is defined as a “*Change in a person’s perception, cognitive function or mood which negatively influences their capacity to function as they would typically choose.*”

Peer Worker: Peer workers includes all workers in mainstream or alternative mental health services or initiatives who are employed to openly identify and use their lived experience of mental illness and recovery as part of their work.

Recovery: Recovery is defined as “being able to create and live a meaningful and contributing life in a community of choice with or without the presence of mental health issues” (Citation from Australian Department of Health, A National framework for recovery-oriented mental health services 2013, Chapter 3). It is acknowledged recovery is personal and means different things to different people. In regard to alcohol and other drug use, it may or may not involve goals related to abstinence (WA MH AOD Services Plan 2015-2025, WA MHC).

Therapeutic Relationship: The therapeutic relationship refers to the relationship between a healthcare professional and a consumer or patient that is professional, caring, respectful of personal boundaries, transparent and focuses positively on interactions that empowers individuals to actively participate in care planning and goal setting.

Therapeutic engagement: Therapeutic engagement involves healthcare professionals spending quality time with patients and aims to empower the patient to actively participate in their care. The consumer experiences their interaction with the therapist as one that is safe, compassionate and non-judgemental (Pereira & Woollaston, 2007).

Trauma Informed: Trauma-informed’ services are aware of and sensitive to the dynamics of trauma as distinct from directly treating trauma per se and recognise that consumers are more likely to be re-traumatised when they encounter experiences that trigger memories of past trauma.

5. Principles

Community Mental Health Services (MHS) will ensure that services provided:

- are patient centred and trauma informed;
- are recovery oriented with an emphasis on collaborative care planning;
- respect diversity and are sensitive to the needs of individuals, families, carers and communities;
- seek cultural consultation for Aboriginal and Torres Strait Islander (ATSI) and Culturally and Linguistically Diverse (CaLD) consumers and provide access to interpreter services when required (including AUSLAN) and information in alternative languages where available;
- respect the needs of Aboriginal people and provide access to Aboriginal Health Workers to work with individuals;
- provide strategies to ensure sexual safety for consumers in accordance with the Chief Psychiatrist's Sexual Safety Guidelines;
- recognise individual vulnerability and are responsive to the needs of individuals, families, carers and communities
- seek specialist input, advice and information from other external specialists as required for example Gender Diversity Services and LGBTQI+

-
- are delivered by clinicians with the range of skill sets required to provide therapeutic interventions and treatments in least restrictive environments in accordance with the requirements of the Mental Health Act (MHA) 2014.

All staff working in Community Mental Health Services are required to adhere to this policy.

Consumers must be consulted and included in the decision to provide their personal information to individuals, agencies or other services. If there are concerns or requests to release personal information, this should be discussed with the consumer and the nominated person, primary carer/personal support person, and attempts made to resolve the concerns.

If it is considered that there is significant risk to the consumer and/or others, then information may be provided to a nominated person, carer, close family member or other personal support person without explicit consumer consent where the clinician's duty of care takes priority over the consumer's wishes. Any decision to disclose personal information without the consent of the consumer should be discussed with the relevant medical officer and the outcome documented in the medical record. Refer to [Chief Psychiatrist Clinicians' Practice Guide to the Mental Health Act 2014](#).

Admission and Discharge planning must occur in collaboration with the consumer, nominated person, carer, close family member (or other personal support person), GPs, other health services, and/or advocate/s to ensure effective transition between treatment settings to reduce the risk of relapse and ensure continuity of care.

6. Procedures

Community Mental Health Services will provide the following:

- All consumers will have a standardised Access, Entry, Case Management and Discharge procedures that include assessment and discharge processes in accordance with the Mental Health Act 2014, the [Triage to Discharge, Mental Health Framework for State-wide Standardized Clinical Documentation](#), (SSCD) the [National Standards for Mental Health Services Standards](#) (NSMHS) and the National Safety and Quality Health Service (NSQHS) Standards. Access, entry, case management and discharge procedures that are in accordance with [NMHS MH Clinical Risk Assessment and Management Policy](#), the [MH Physical Healthcare Management Policy](#) and the [NMHS Clinical Documentation Policy](#).
- The identification of an allocated Case Manager (community) or other delegated person responsible for coordinating care and informing the consumer and carer, close family member (or other support person identified by the consumer)) about the management plans, changes in the management plan and any referrals or discharge plans initiated to ensure adequate information is available to support the consumer and maintain continuity of care (Refer to [SSCD](#) documentation requirements, the [NMHS Clinical Documentation Policy](#) and the [Chief Psychiatrist Clinicians' Practice Guide to the Mental Health Act 2014](#)).
- All consumers will have a comprehensive care plan formulated by a clinician in consultation with the multidisciplinary care team including the Consultant Psychiatrist or Psychiatrist Registrar/Medical Officer and in collaboration with the consumer and carer, close family member or other personal support person. The consumer is to co-sign their



care plan and copies are to be provided to both the consumer and nominated carer, close family member or other personal support person.

- A comprehensive risk assessment in line with best practice guidelines, as indicated in the [NMHS MH Clinical Risk Assessment and Management Policy](#). Assessments must consider individual and situational risk, environmental risks and the availability of supports and other protective factors.
- Assessments inclusive of mental and physical health assessments, including oral health checks, alerts (diabetes, medication sensitivity, infection, self-harm, suicide, immunocompromised status, substance use, aggression, falls or pressure injury risks are undertaken in accordance with the requirements of the [SSCD](#) documentation [NMHS Falls Risk Assessment and Management Policy](#), [MH Medication Prescribing, Administration and Management in the Community](#), WA Health [Clinical Care of People Who May Be Suicidal Policy](#) and the WA Health [Clinical Care of People With Mental Health Problems Who May At Risk of Becoming Violent or Aggressive Policy](#).

6.1 Admission Procedures

Legislative and policy requirements for community MHS require that:

- Assessments are completed including a physical health assessment, risk assessment and management plan on admission to the service in accordance with SSCD requirements. (**Youth MH Services refer to Appendix 3 - Youth Mental Health Clinical Review Checklist**)
- Clinical Alerts are updated on PSOLIS and documented in the Medical Record as required ([MH Management of Clinical Alerts Policy](#)).
- The physical health examination can be undertaken (with the consent of the consumer) by the consumer's regular or nominated primary health care provider and recorded in either in the Physical Examination template or in an appropriate format that is provided by for the respective primary health care provider (in accordance with SSCD requirements)
- Clinicians will encourage and support consumers to identify a doctor/gp and to schedule and attend regular appointments to manage their physical health care needs.
- Services have defined pathways for admission and discharge with appropriate service allocation, (e.g. triage and emergency assessment, referral processes, case management allocation, discharge processes)
- Procedures and processes facilitate access and are designed to minimise delays and duplication in assessment (includes physical, mental health and risk assessment), treatment, care planning and care delivery, recovery planning, and discharge planning.
- Clinicians encourage and support consumers, families, and carers to actively engage in all aspects of care including, assessment, treatment planning, crisis awareness planning and discharge planning, if appropriate.
- Management plans including risk are reviewed at three monthly intervals and as part of the discharge planning process in accordance with the requirements of the SSCD.



- Clear expectations regarding the potential length of treatment within community services must be provided on admission and throughout the period of active treatment with a predicted discharge date identified to assist consumers, families and carers understand treatment goals and the role of community services in supporting on going recovery. With the exception of specialist community services where the length of treatment is related to individual needs.
- Clinicians provide consumers, families, and carers with specific information with regard to escalating care in the event of physical deterioration, mental health deterioration and escalation of risk that includes emergency contacts and response services that are accessible 24/7.

6.2 Discharge Procedures

- Admission and Discharge planning is an integral part of the continuum of care that commences as early as possible within the episode of care and focuses on coordinating services effectively so that discharge occurs seamlessly, and all parties are consulted and kept informed of the discharge plan. [Refer to MH Discharge Planning for Adult Community Consumer Procedure](#).
- All clinicians involved in delivering care have responsibility for review of treatments provided and for participation in the discharge process.
- The [Treatment, Support and Discharge Plan](#) (TSDP) (SMHMR907) /PSOLIS Management Plan is developed with each consumer and where relevant the nominated personal support person in accordance with the requirements of the [DoH Triage to Discharge Mental Health Framework for SSCD](#), and is utilised and updated throughout the consumer's admission to MH services. This document identifies discharge requirements from the point of admission and is utilised as the final document outlining discharge plans for the consumer prior to discharge from the service.
- Discharge Plans are presented by the case manager at the multidisciplinary team (MDT) meetings and include preparation for discharge, risk/s identified, ongoing care arrangements and discussion of referrals and post discharge supports required
- The TSDP/PSOLIS Management Plan is reviewed by the consumer and where appropriate, signed by the consumer and case manager to indicate agreement. If the consumer declines or is unable to sign the clinician will document on the plan and in the medical record/progress notes. The TSDP/ /PSOLIS Management Plan is reviewed with the consumer (and relevant support person at the request of the consumer) **at three monthly intervals** in accordance with the [SSCD](#) requirements. The TSDP/ /PSOLIS Management Plan includes identification of follow up arrangements and contact with GP and community managed organisations.
- Discharge plans are discussed with the consumer, family and carers taking into consideration cultural and diversity needs of the consumer (e.g. for Culturally and Linguistically Diverse, LGBTQI+). MH clinicians actively engage with specialist services such as an Aboriginal Mental Health Professional, CaLD consultant, Aboriginal Health Services or external specialist agencies prior to discharge to facilitate transitional support and post discharge follow-up arrangements where appropriate.

- All discharges, including consumers who decline a follow up service are discussed by the multidisciplinary care team including the Consultant Psychiatrist or Psychiatrist Registrar/Medical Officer, where. All clinicians should ensure they are familiar with follow the processes set out in the [NMHS MH Consumer Declining Assessment or Follow-up Policy](#), prior to discharging consumers who decline any follow up or post discharge contact with services.
- On discharge, the [Treatment, Support and Discharge Plan](#) (SMHMR907) / Management Plan is updated and discussed with the consumer and personal support person. This should be signed by the consumer and personal support person where relevant to corroborate understanding of the plan. If the consumer refuses to sign the management plan, this should be documented in the medical record along with the date and time of the discussion. A copy of the plan is provided to the consumer, personal support person and GP, with the consent of the consumer as appropriate.
- Clinicians provide consumers, families, and carers where relevant with verbal and written information in relation to services provided, treatment received including specific information relating to medications, referrals and discharge planning. This exchange of information frequently occurs over a number of meeting and allows time for clinicians to clarify issues and respond to queries that consumers and support persons have with regard to discharge or transfer of care.

Minimum requirements for Community MHS when referring consumers for ongoing care to other services:

- Referral is sent to the service and contact is made to confirm outcome of the referral.
- When a referral is accepted, contact is made with the identified contact person to discuss the care transfer as agreed with the consumer.
- Where possible a meeting is arranged to include the consumer so that the consumer can be introduced to the identified contact person in the receiving service and begin the process of engagement.
- The receiving service is provided with a fully completed care transfer summary or letter as detailed in the [Chief Psychiatrist Clinicians' Practice Guide to the Mental Health Act 2014](#).
- The receiving service is provided with a copy of the negotiated/signed [Treatment, Support and Discharge Plan](#) (SMHMR907) / PSOLIS Management Plan when appropriate, and as agreed with the consumer.

Clinical records will be finalised on discharge by:

- Recording completion of all discharge requirements maintained in the medical records and for [Community Adult](#) utilising the NMHS MH Community Adult Discharge Checklist sticker (**Appendix 1**) and the Community Adult Discharge Checklist (**Appendix 2**).
- Recording completion of all discharge requirements maintained in the medical records and for Youth Mental Health completing the Discharge Clinical Review within the Youth Mental Health Clinical Review Checklist (**Appendix 3**)

Discharge documentation includes the following:

- Completed HONOS on discharge
- [Risk Assessment and Management Plan \(RAMP\)](#) (SMHMR 905)

- Identified that consumer and where relevant the personal support person has been informed of the discharge plan and date of discharge
- Provision of the [Treatment, Support and Discharge Plan](#) (SMHMR907) to the consumer and relevant others as agreed with the consumer
- Information regarding discharge medications and how to access these post discharge, especially if there is delay before the first face to face meeting with the GP/service.
- Details of communication with original referrer and/or GP with discharge medications and follow up arrangement provided
- Completion of a formal Discharge Letter or Transfer Care Summary
- Details of whom the Discharge Summary was provided to
- Details of discharge address and phone number for the consumer
- Details of emergency contacts in the event of an emergency, clinical deterioration or issue of concern

The Community MHS/case manager provides the consumer and personal support person/s with information on the process for facilitating re-entry for service and resources for crisis contact.

7. Collaborative Care Planning

Consumers, families and carers where relevant must be provided with the following information on entering the service and at regular intervals while they receive treatment.

An explanation of entry processes, supports available and specific information about the services that consumers, families and carers can expect to receive on entering and engaging with mental health services, as identified in the [National Safety and Quality Healthcare Standards \(NSQHS\)](#) and [the Chief Psychiatrist's Standards for Clinical Care \(2015\)](#).

An explanation of exit processes, including information that consumers, families and carers can expect to receive on discharge or transfer for ongoing care and treatment in accordance with SSCD requirements:

- Information about ongoing access to Community MHS, hours of service, contact numbers, after hours emergency, contacts, case manager allocation, home visiting, and collaborative care planning and clinical documentation (compliant with the SSCD requirements).
- A verbal explanation of the consumer's legal status in accordance with the MHA 2014, and rights and responsibilities as per the [National Charter of Healthcare Rights](#) and the MHA, and about the treatment that is provided to the consumer.
- A list of 'out of hours' contact numbers, community services and other support services information, including emergency response services available.
- Information about consumer advocacy and other supports available to assist the consumer to understand what is happening for them and assist them in identifying their goals and participate in their care planning.
- Information about transitional care, discharge planning and referral processes to appropriate specialist clinicians and services, such as Physicians and Specialists, GP Services, Metabolic Screening & Monitoring Services, Clozapine Clinic, Alcohol and other Drug services, Centre for Clinical interventions, Hospital in the Home, Silver Chain,

Mental Health Rehabilitation, Living Skills Programs, Accommodation Services, Social Services and NDIS if appropriate.

- Information about in-reach services for mental health consumers admitted to a general hospital for treatment.

8. Involvement of Family, Carers and Relevant Others

With consent of the consumer, identified nominated carer, close family member or identified support person must be provided with:

- Information about relevant services to assist them in understanding the consumer's condition, signs and triggers of relapse, and services for advice and support, particularly for times of crises.
- The name and contact details of the allocated Case Manager, Consultant Psychiatrist and/or medical member of the team responsible for the care and treatment that the consumer receives.

Care Planning must also include:

- Request for and documentation of the consumer's informed consent for the inclusion of, and release of information to the carer, close family member or other personal support person or advocate.
- Involvement of the carer, close family member or other personal support person, especially in life-threatening situations. Consumer confidentiality should not be used as a reason for not communicating with carers in these situations (see part 9 "Notifiable Events" of the MHA).

9. Documentation

Mental health care is especially dependent on data gathering and accurate clinical documentation. Assessment and diagnosis require detailed information often obtained from multiple sources requiring organisation and formulation. Compliance with the SSCD suite of documents is mandatory.

Completion of clinical documentation on PSOLIS and in the Medical Record in accordance [SSCD](#) requirements and the [NMHS Clinical Documentation Policy](#) with the recording of the consumer nominated carer, close family member or other personal support person's name and contact details in WebPAS, Psychiatric Services On-Line System (PSOLIS) and in the medical record.

Care is provided by a multidisciplinary team of clinicians, case managers and specialist teams and the following strategies have been identified as essential to strengthen the safety and quality of clinical documentation and to ensure that the consumer and their family, and carers are actively involved in all aspects of care planning.

Completion of the relevant validated assessment tools, the State-wide Standardised Clinical Documentation (SSCD) suite and relevant PSOLIS forms including transfer of information for discharge and timely completion of discharge summaries.

On admission, throughout treatment and prior to discharge, the care plan must be discussed in a way that the consumer understands and be signed by the consumer to acknowledge receipt of the plan.

Every consumer must have a TSDP (SMHMR907)/ PSOLIS Management Plan completed and receive a copy of it prior to discharge. The TSDP/PSOLIS Management Plan must accompany the consumer between community and other treatment settings and be communicated to new clinicians and relevant others at the time of transition. This facilitates the transfer of pertinent information between key care and continuation of treatment as planned

The family/carer are involved in the discharge planning process and in the development of the discharge plan, as appropriate. The consumer's TSDP/PSOLIS Management Plan needs to be accurately communicated over time.

The assessment, care planning and decision to refer a consumer from one public mental health service to another should be a seamless transition and the consumer should not experience further assessments as a barrier to entry.

All clinical incidents are recorded in DatixCIM and on PSOLIS and the Patient Medical Records are updated as soon as possible in accordance with [NMHS Clinical Documentation Policy](#).

10. Clinical Handover

Clinicians must use the iSoBAR clinical handover process when communicating with members of the multidisciplinary team, intake and referral services and when engaging in admission and discharge planning in accordance with the [DoH Clinical Handover Policy and Guidelines](#).

Transfer of care documentation is completed, and iSoBAR clinical handover process followed in accordance with the requirements of the [DOH Clinical Handover Policy and Guidelines](#), the [NMHS Clinical Documentation Policy](#) and [SSCD](#) requirements.

11. Compliance and Evaluation

Compliance measures to be met in relation to completion of clinical documentation on admission and discharge as identified in SSCD and NOCC requirements and audited by the service as part of the SQP Audit Calendar include the following:

- 100% of active consumers have a completed [Treatment, Support and Discharge Plan](#) (SMHMR907)/ PSOLIS Management Plan in the clinical file.
- 100% of consumers have signed their TSDP/PSOLIS Management Plan with a copy placed in the Medical Record.
- 100% of discharge clinical records have completed discharge requirements.
- Copy of Discharge letter / care transfer summary is sent to GP / receiving service.
- 100% clinical records indicate involvement of, and information provided to family/carer personal support person with consent of the consumer (or documentation of reasons why not informed).
- 100% of consumers have a discharge [RAMP](#) (SMHMR905) in file and on PSOLIS.
- 100% of active consumers have a discharge NOCC completed in PSOLIS and printed in file.
- 100% of nominated carers, identified close family members or other support persons receive identified information throughout the episode of treatment in the service,

including updates on the management plan and discharge planning (with the consent of the consumer).

- 100% of contact details for consumers family member, carer and/or other support person's to be entered into Topas/PSOLIS.
- Leadership Committees receive data reports and audit results and recommendations for their services to review
- Training Compliance data is monitored and reported to MHPHDS Executive (e.g. PSOLIS, MHA 2014)

Audit reports are compiled by SQPU and disseminated to Leadership committees to identify action plans and initiate service improvements.

Related internal policies, procedures and guidelines

[NMHS MH Case Management Policy](#)

[NMHS MH Medication Prescribing, Administration and Management in the Community Policy](#)

[NMHS MH Clinical Risk Assessment and Management Policy](#)

[NMHS MH Management of Clinical Alerts Policy](#)

[NMHS Clinical Documentation Policy](#)

[NMHS MH Consumer Declining Assessment or Follow-up Policy](#)

[NMHS MH Did Not Attend \(DNA\) Community Appointment Procedure](#)

[NMHS MH Community Adult After-Hours Clinical Handover Procedure](#)

[NMHS MH Discharge Planning for Adult Community Consumer Procedure](#)

References

[WA Health Code of Conduct Policy \(MP 0124/19\)](#)

[Triage to Discharge. Mental Health Framework for State-wide Standardized Clinical Documentation \(SSCD\)](#)

[WA Health Clinical Care of People Who May Be Suicidal Policy](#)

[WA Health Clinical Care of People With Mental Health Problems Who May At Risk of Becoming Violent or Aggressive Policy](#)

[Mental Health Act 2014 WA](#)

[National Standards for Mental Health Services 2010](#)

[National Safety Quality Healthcare Standards](#)

[Chief Psychiatrist's Standards for Clinical Care 2015](#)

[Chief Psychiatrist Clinicians' Practice Guide to the Mental Health Act 2014](#)

[DoH Community Mental Health Status Assessments: Role of Mental Health Clinician Policy 2021 MP 0099](#)

[DOH Clinical Handover Policy](#)

[DoH Clinical Handover Guidelines](#)

Useful resources

[Clinical Governance, Safety and Quality Policy Framework](#)

[Risk Assessment and Management Plan \(RAMP\) \(SMHMR905\)](#)

[Treatment, Support and Discharge Plan \(SMHMR907\)](#)

ACSQHC Standards Guide for Community Health Services

SSCD Forms

NMHS MHPHDS Community Adult Discharge Checklist sticker

Admission and Discharge Checklist (Community) (PMR 05)

Policy Sponsor	Director of Clinical Services MHPHDS				
Policy Contact	Co Director of Community Mental Health Services MHPHDS				
Date First Issued:	20/01/2023	Last Reviewed:	N/A	Review Date:	20/01/2026
Approved by:	MH Policy Coordination Committee			Date:	03/08/2022
Endorsed by:	Director of Clinical Services MHPHDS			Date:	28/10/2022
NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☒  Std 2: Partnering with Consumers ☒  Std 3: Preventing and Controlling Healthcare Associated Infection ☒  Std 4: Medication Safety		☒  Std 5: Comprehensive Care ☒  Std 6: Communicating for Safety ☐  Std 7: Blood Management ☒  Std 8: Recognising and Responding to Acute Deterioration		
National Standards for Mental Health Services	☐ Std 1: Rights and Responsibilities ☒ Std 2: Safety ☐ Std 3: Consumer and Carer Participation ☐ Std 4: Diversity Responsibility ☐ Std 5: Promotion and Prevention ☐ Std 6: Consumers ☐ Std 7: Carers ☐ Std 8: Governance, leadership and management		☐ Std 9: Integration ☒ Std 10: Delivery of Care ☐ 10.1 Supporting Recovery ☐ 10.2 Access ☐ 10.3 Entry ☐ 10.4 Assessment and Review ☐ 10.5 Treatment and Support ☐ 10.6 Exit and Re-entry		
Chief Psychiatrist's Standards for Clinical Care:	Aboriginal Practice, Assessment, Care Planning, Consumer and Carer Involvement in Individual Care, Physical Health Care of Mental Health Consumers, Risk Assessment and Management, Transfer of Care				
Printed or personally saved electronic copies of this document are considered uncontrolled					

Document Version Control			
Date	Version	Author	Notes
20/01/2023	1.0	Director of Clinical Services MHPHDS	To effectively identify admission and discharge processes to facilitate access to community mental health services for consumers, families and carers and provide timely and planned discharge to appropriate services as required in accordance with individual needs and identified risk.

The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative (ISD 1794).

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Appendix 1:MHPHDS Community Adult Discharge Sticker (sample only)

MHPHDS Community Adult Discharge Checklist	
SERVICE REQUIREMENTS	
Notify	☐ Consumer & Support Person/Carer
	☐ Administration as per service requirements
Update	☐ CAPlan
	☐ RAMP
	☐ Care Plan
Complete	☐ Discharge NOCC (HoNOS, LSP-16, K10+)
	☐ Discharge entry in medical record
	☐ PSOLIS Admission, Discharge, Transfer & Referral Form
	☐ Care Transfer Summary (PMHMR916)
CONSUMER & SUPPORT PERSON/CARER REQUIREMENTS	
Provide	☐ Information regarding re-entering the service
	☐ Information regarding emergency services
	☐ Copy of CAPlan
	☐ Copy of Care Plan
	☐ Copy of Coping and Awareness Tool (optional)
	☐ Copy of Care Transfer Summary (PMHMR916) if appropriate
	☐ Medication information as required
	☐ Follow up arrangements as required
Notify original referrer and/or GP	
Or	☐ Letter / Phone Call
	☐ Copy of Care Transfer Summary (PMHMR916) if appropriate
Discharge address & phone number	
Completed by	
Signed	
Dated	

Appendix 2 :MH Admission and Discharge Checklist (Adult Community) (PMR 05)

NORTH METROPOLITAN HEALTH SERVICE MENTAL HEALTH Clinic ADMISSION AND DISCHARGE CHECKLIST (COMMUNITY)		Surname		Sex	U.R. No
		Forenames		D.O.B.	
		Address			
		Ward	Registrar	Consultant	
		Use Patient I.D. Label when available			
ADMISSION CHECKLIST					
Date:				Signature	
DOCUMENTATION					
Complete SSCD MENTAL HEALTH ASSESSMENT FORM (SMHMR902)					
Complete NOCC (HoNOS, LSP-16, K-10+) on PSOLIS					
Complete PSOLIS Management Plan/Treatment, Support and Discharge Plan (SMHMR 907) with Consumer; Consumer to sign and be supplied with a copy.					
Complete Brief Risk Assessment (BRA)/Risk Assessment and Management Plan (RAMP) (SMHMR 905)					
Complete AUDIT and DAST					
Give Crisis Awareness Plan to Consumer and encourage to complete					
Commence Physical Examination Form (SMHMR903)(if no evidence that GP had conducted within 12 months) and					
Complete Physical Appearance Form (SMHMR 904)					
Discuss Release of Information Form , and Consumer to sign					
(Only if Consumer over 65, 50 if Aboriginal, or at risk of falls) Complete Falls Risk for Older People (FROP) and educate Consumer re falls					
PROCESS					
Inform Admin of admission					
Inform Referrer, and/or GP and Next of Kin/Carer of admission to service					
Document Next of Kin/Carer, GP and other involved person/agency on PSOLIS and PMR2 Personal Contact Form					
Allocate Case Manager and enter details on PSOLIS					
Add any necessary Alerts to PSOLIS and complete Clinical Alert Form					
Print out and File PSOLIS TRIAGE FORM/ SSCD TRIAGE FORM (SMHMR 900)					
Provide Consumer and Carer with information pack (including information on how to access/reaccess services) and document in medical record to indicate this has been completed					
Consumer provided with an explanation of or access to information regarding Consumer Charter of Rights					
If DCP Involvement required, consider referral to Social Worker					
Identify if any CALD issues					
Confirm that the Consumer and Carer has been provided with information on any medications prescribed					

PMR 05 ADMISSION AND DISCHARGE CHECKLIST (COMMUNITY)

HCHGREFMR005A

G151
04/15

Appendix 2 :MH Admission and Discharge Checklist (Adult Community) (PMR 05)

NORTH METROPOLITAN HEALTH SERVICE MENTAL HEALTH Clinic ADMISSION AND DISCHARGE CHECKLIST (COMMUNITY)	Surname		Sex	U.R. No
	Forenames		D.O.B.	
	Address			
	Ward	Registrar	Consultant	
	Use Patient I.D. Label when available			
DISCHARGE CHECKLIST				
Date:			Signature	
DOCUMENTATION				
Complete a Care Transfer Summary (SMHMR 916) and ensure a copy is given to Consumer and Carer				
Update PSOLIS Management Plan/ Treatment, Support and Discharge Plan (PMHMR 907)				
Complete Brief Risk Assessment (BRA)/Risk Assessment and Management Plan (RAMP) (SMHMR 905)				
Complete Discharge NOCC on PSOLIS				
PROCESS				
Inform Admin of: Time, Address (ensure correct discharge address is given), Status on discharge, Diagnosis & Follow up arrangements				
Notify Next of Kin				
Notify original referrer and/or GP				
Check follow up has been organised if required and Consumer is aware of arrangements.				
Give Consumer and Carer emergency contacts card.				
Provide Consumer with information on any medications prescribed				
Complete discharge entry in progress notes (including MSE)				
Consumer and Carer to receive a copy of their Treatment Support and Discharge Plan/PSOLIS Discharge Plan, Coping and Awareness Tool, Crisis Awareness Plan				
Did the Consumer and Carer receive Consumer Information Package (including information on how to access/reaccess services) on admission? If no, please supply.			Yes	
			No	

Appendix 3: Youth Mental Health Clinical Review Checklist (page1)



Government of Western Australia
North Metropolitan Health Service
Mental Health, Public Health and Dental Services

Youth Mental Health Clinical Review Checklist

Youth Site _____

Surname	UMRN
Forename	
Gender	DOB
Case Manager	

Please check off each item below and file in the front of the medical record.

First Session Tasks

Date of Session: _____

Task	Yes	If no, please comment
Welcome Pack provided and discussed with Client and Carer (where indicated)		
Consent for Treatment and Release of Information signed (indicate if mature minor status assessed)		
* ASSCD Risk Assessment and Management Plan (RAMP) is completed and filed in the medical record		
Letter to referrer sent and documented in file (copy in correspondence)		
Ngatti Clients Only – Signed consent to share information between Ngatti and YouthReach South		

Clinician signature: _____ Team Leader signature: _____ Date: _____

Initial Assessment/Activation review

Date Presented: _____

Task	Yes	If no, please comment
Activation form completed and provided to administration		
A Mental Health Assessment (SMHMR902) has been completed and is in medical record *		
Has there been communication with the young person's General Practitioner? *		
A Family, Friend or Carer to support the young person's care, has been identified by the young person and recorded in the medical record		
All NOCC (K10 / LSP/ HoNOS) measures have been completed with valid responses		
A PSOLIS Individual Management Plan is completed and is in the medical record *		
The draft Management Plan has been discussed with the client and will be signed, with copy provided to the Client (and Carer where indicated) *		
Discharge Planning commenced with Client and Carer and documented in medical record. If clinically inappropriate this is documented in medical record. *		
PSOLIS alerts reviewed and entered/removed, where indicated and documented on Clinical Alert Form PMR01.		Alert list has to be printed in front of file with a completed Clinical Alert Risk PMR01
Diagnosis entered on PSOLIS (documented on Activation form)		Primary: Secondary:
For clients transferred to another mental health service or other service provider the medical record contains a completed SSCD Care Transfer Summary *		
Update Contact sheet and PSOLIS details		
Research Consent Form offered/signed		

Clinician signature: _____ Team Leader signature: _____ Date: _____

* Stokes Review Recommendation item

Appendix 3: Youth Mental Health Clinical Review Checklist (page 2)



Government of Western Australia
North Metropolitan Health Service
Mental Health, Public Health and Dental Services

Youth Mental Health Clinical Review Checklist

90 Day Clinical Review Date Presented: _____

Surname	UMRN
Forename	
Gender	DOB
Case Manager	

Task	Yes	If no, please comment
A Brief Risk Assessment or completed SSCD Risk Assessment Management Plan is completed in the last 90 days and is placed in medical record *		
Has there been communication with the young person's General Practitioner? *		
A Family, Friend or Carer to support the young person's care, has been identified by the young person and recorded in the medical record		
All NOCC (K10 / LSP/ HoNOS) measures been completed with valid responses		
The PSOLIS Individual Management Plan is reviewed, updated and is in the medical record *		
The previous Management Plan has been signed and provided to the Client (and Carer where indicated) *		
Discharge Planning undertaken with Client and Carer documented in medical record (if not undertaken at assessment). If clinically inappropriate this is documented in medical review. *		
PSOLIS alerts reviewed and entered/removed, where indicated and documented on Clinical Alert Form PMR01.		Alert list must be printed in front of file with a completed Clinical Alert Risk PMR01
Diagnosis reviewed and entered on PSOLIS		Primary: Secondary:
For clients transferred to another mental health service or other service provider, the medical record contains a completed SSCD Care Transfer Form *		
Update Contact sheet and PSOLIS details		
Research consent form offered/signed		
ESQ offered to young person and carer (where indicated)		

Clinician signature: _____ Team Leader signature: _____ Date: _____

* Stokes Review Recommendation item 12 June 2018
October 2021 – Youth Mental Health Clinical Review checklist for clinical staff

Appendix 3: Youth Mental Health Clinical Review Checklist (page 3)



Government of Western Australia
North Metropolitan Health Service
Mental Health, Public Health and Dental Services

Youth Mental Health Clinical Review Checklist

Discharge Clinical Review Date presented: _____

Surname	UMRN
Forename	
Gender	DOB
Case Manager	

Task	Yes	If no, please comment
An updated Brief Risk Assessment or completed SSCD Risk Assessment Management Plan is in medical record*		
Has there been communication with the young person's General Practitioner?*		
A Family, Friend or Carer to support the young person's care, has been identified by the young person and recorded in the medical record		
Deactivation NOCC (K10 / LSP / HoNOS) measures have been completed with valid responses		
A final PSOLIS Individual Management Plan is completed and is in the medical record*		
The Management Plan has been signed and provided to the Client (and Carer where indicated)*		
Discharge Planning with client and carer involvement. Notified of closure, relapse prevention strategies, and re-entry strategies discussed and documented. MHERL brochure provided if appropriate*		
PSOLIS alerts reviewed and entered/removed, where indicated		
Referrer and other stakeholders notified of discharge, where appropriate.		
Diagnosis reviewed and entered on PSOLIS		Primary: Secondary:
The Care Transfer Form (SSCD) has been completed and forwarded to stakeholders and filed in the medical record*		
Progress notes completed		
Update Contact sheet and PSOLIS details		
Research Consent Form offered/signed		
ESQ offered to young person and carer (where indicated)		
Deactivation form completed and provided to administration		

Clinician signature: _____ Team Leader signature: _____ Date: _____

* Stokes Review Recommendation item 12 June 2018
October 2021 – Youth Mental Health Clinical Review checklist for clinical staff